

Infant Syrup

120mg/5ml (Orange)

Uphamol 120



PHARMACODYNAMICS

Analgesic: The mechanism of analgesic action has not been fully determined. Paracetamol may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (CNS) and to a lesser extent, through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or action of other substances that sensitize pain receptors to mechanical or chemical stimulation.

Antipyretic: Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

PHARMACOKINETICS

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract following oral administration. Approximately 90 to 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulfuric acid and cysteine. An intermediate metabolite, which may accumulate in overdosage after the primary metabolic pathways become saturated, is hepatotoxic and possibly nephrotoxic. The half-life of paracetamol, 1 to 4 hours following oral administrations, does not change with renal failure but may be prolonged in acute overdosage, in some forms of hepatic disease, in the elderly, and in the neonate. However, it may be somewhat shortened in children. The time for paracetamol to reach its peak concentration is 0.5 to 2 hours and time needed to reach the peak effect is 1 to 3 hours. Duration of action is 3 to 4 hours. It is excreted primarily as conjugates through renal and only 3% of a dose may be excreted unchanged.

INDICATION

Used to relieve fever and mild to moderate pain associated with conditions such as aches due to cold and flu, teething, post vaccination reactions, headache, earache and toothache.

DESCRIPTION

An orange, fruity flavoured syrup.

Each teaspoonful (5ml) contains 120mg acetaminophen (paracetamol).

Preservatives:

Methylparaben 0.21% w/v

Propylparaben 0.031% w/v

DOSAGE

Below 3 months	5 to 10mg/kg (On doctor's advice only)
3 to 11 months	1/2 to 1 teaspoonful (2.5 to 5ml), 3 to 4 times daily
1 to 5 years	1 to 2 teaspoonfuls (5 to 10ml), 3 to 4 times daily

Do not take more than 4 times in 24 hours and never exceed the recommended dose unless prescribed by your doctor or pharmacist.

SIDE EFFECTS

When taken as directed, unwanted effects from paracetamol are rare and usually mild. Skin rashes and other hypersensitivity reactions have been reported occasionally.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

CONTRAINDICATION

Uphamol 120 should not be taken if children are allergic to acetaminophen (paracetamol).

WARNINGS/ PRECAUTIONS

If symptoms persist, please consult your doctor or pharmacist.

Do not keep outdated Uphamol 120.

Allergy alert: Paracetamol may cause severe skin reactions.

Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition.

If these reactions occur, stop use and seek medical assistance right away.

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time.

INTERACTIONS WITH OTHER MEDICAMENTS

Drugs which induce the specific microsomal isoenzyme responsible for the metabolic activation of paracetamol might be expected to increase susceptibility to its toxic effects on the liver humans as in the case in animals.

The absorption of paracetamol may accelerated by drugs such as metoclopramide. Excretion may be affected and plasma concentrations altered when given probenecid. Colestyramine reduces the absorption of paracetamol if given within 1 hour of paracetamol. Drug interaction also reported with warfarin, rifampicin, chloramphenicol, antiepileptic (carbamazepine, phenorbital, phenytoin, or primidone), antiviral (interferon alfa and zidovudine) and probenecid.

PREGNANCY AND LACTATION

Paracetamol crosses the placenta but the drug has been used widely as an analgesic in pregnancy and no adverse fetal effects has been recorded.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

Sign and Symptoms of Overdose

Vomiting, mild nausea, gastrointestinal upset, hypotension, anorexia and diarrhoea.

TREATMENT (For medical practitioner's information)

- Empty the stomach via induction of emesis or gastric lavage.
- Acetylcysteine administration is recommended to be instituted immediately after ingestion of an overdose has been reported without waiting for the results of urine and plasma level tests. Acetylcysteine is most effective if treatment is started within 10 to 12 hours after ingestion of overdose. However, it may be of some benefit if treatment is started within 24 hours. For oral administration, the recommended adult dose of acetylcysteine is 140mg/kg initially, then 70mg/kg every 4 hours for 17 doses. Each dose should be diluted to a 5% solution of drinking fluid prior to administration because of its unpleasant odour and its sclerosing properties. Any dose vomited within 1 hour of administration must be repeated. If necessary, the antidote may be given via duodenal intubation.
- Plasma acetaminophen concentration should be determined at least 4 hours following the ingestion of overdose in order to ascertain the status of patient. Renal and cardiac function should be monitored and appropriate therapy instituted if necessary.

Up-to-date information of treatment of overdose can be obtained from The National Poison Centre, University Sains Malaysia, freephone no: 1-800-88-8099 (Monday to Friday: 8.30am - 4.30pm, Saturday: 8.30am - 1.00pm)

PACKING

In bottles of 60ml and 120ml.

Not all pack sizes are available locally.

STORAGE

KEEP CONTAINER TIGHTLY CLOSED.
STORE IN A DRY PLACE BELOW 30°C.

PROTECT FROM LIGHT.

KEEP OUT OF REACH OF CHILDREN.

Route of administration: Oral.

SHELF LIFE

Please refer to box or label.

Sirap Bayi

120mg/5ml (Oren)



Uphamol 120

FARMAKODINAMIK

Analgesik: Mekanisme tindakan analgesik belum ditentukan sepenuhnya. Paracetamol boleh bertindak terutamanya dengan menghalang sintesis prostaglandin di sistem saraf pusat (CNS) dan pada tahap yang lebih rendah, melalui tindakan periferil dengan menyekat penghasilan rasa sakit. Tindakan periferil juga mungkin disebabkan perencatan sintesis prostaglandin atau perencatan sintesis atau tindakan bahan lain yang memeka reseptor kesakitan kepada rangsangan mekanikal atau kimia. **Antipiretik:** Paracetamol mungkin menghasilkan antipireksis dengan bertindak secara berpusat pada pusat pengatur suhu hipotalamus untuk menghasilkan vasodilatasi periferil yang mengakibatkan peningkatan aliran darah ke kulit, berpeluh, dan kehilangan haba. Tindakan pusat mungkin melibatkan penghambatan sintesis prostaglandin di hipotalamus.

FARMAKOKINETIK

Paracetamol cepat dan hampir sepenuhnya diserap dari saluran gastrousus selepas pemberian oral. Lebih kurang 90 hingga 95% daripada dos adalah dimetabolisme di hati, terutamanya dengan konjugasi dengan asid glukuronik, sulfurik asid dan sistein. Metabolit perantara, yang mungkin terkumpul di dos berlebihan selepas laluan metabolik utama menjadi tepu, adalah hepatotoksik dan mungkin nefrotoksik. Separuh hayat paracetamol, 1 hingga 4 beberapa jam selepas pemberian oral, ianya tidak berubah dengan kegagalan buah pinggang tetapi mungkin berpanjangan dalam dos berlebihan akut, dalam beberapa bentuk penyakit hati, dalam kalangan orang tua, dan pada bayi yang baru lahir. Walau bagaimanapun, ia mungkin dipendekkan dalam kalangan kanak-kanak. Masa untuk paracetamol mencapai kepekatan puncak adalah 0.5 hingga 2 jam dan masa yang diperlukan untuk mencapai keberkesanan adalah 1 hingga 3 jam. Tempoh tindakan adalah 3 hingga 4 jam. Ia dikumuhkan terutamanya sebagai konjugat melalui buah pinggang dan hanya 3% daripada dos yang boleh dikeluarkan tidak berubah.

INDIKASI

Digunakan untuk melegakan demam dan kesakitan ringan dan sederhana akibat selsema, pertumbuhan gigi, reaksi selepas vaksinasi, sakit kepala, sakit telinga dan sakit gigi.

DESKRIPSI

Sirap berwarna oren dan berperisa buah. Setiap sudu teh (5ml) mengandungi 120mg acetaminophen (paracetamol).

Bahan Pengawet:

Methylparaben 0.21% w/v

Propylparaben 0.031% w/v

SUKATAN

Bawah 3 bulan	5 hingga 10mg/kg (Atas arahan doktor sahaja)
3 hingga 11 bulan	1/2 hingga 1 sudu teh (2.5 hingga 5ml), 3 hingga 4 kali sehari
1 hingga 5 tahun	1 hingga 2 sudu teh (5 hingga 10ml), 3 hingga 4 kali sehari

Jangan mengambilnya lebih daripada 4 kali dalam jangka masa 24 jam dan jangan melebihi sukatan yang dicadangkan kecuali dinasihatkan oleh doktor atau ahli farmasi anda.

Size: 23.2cm(H) x 15cm(W)

Black 100%

KESAN SAMPINGAN

Apabila digunakan mengikut arahan, kesan sampingan daripada paracetamol adalah jarang berlaku. Radang kulit dan lain-lain reaksi hipersensitif berlaku sekali-sekala sahaja.

Tindak balas hipersensitiviti kulit termasuk ruam kulit, angioedema, Stevens Johnson Syndrome / Toxic Epidermal Necrolysis telah dilaporkan.

PERHATIAN

Uphamol 120 tidak boleh digunakan jika anak anda alah kepada acetaminophen (paracetamol).

AMARAN

Jika sakit berlarutan, sila jumpa doktor atau ahli farmasi anda.

Jangan simpan **Uphamol 120** yang sudah tamat tempoh penggunaannya.

Amaran alahan: Paracetamol boleh menyebabkan reaksi kulit yang teruk. Gejala ini termasuk kemerahan kulit, lepuh atau ruam. Ia boleh menjadi tanda-tanda satu keadaan yang serius.

Jika tindak balas ini berlaku, berhenti menggunakannya dan dapatkan bantuan perubatan dengan segera.

Ubat ini mengandungi PARACETAMOL.
Pastikan anak-anak anda tidak mengambil sebarang ubat sakit kepala, demam dan lain-lain yang mengandungi PARACETAMOL pada masa yang sama.

GEJALA DAN RAWATAN UNTUK DOS BERLEBIHAN

Tanda dan Gejala dos berlebihan

Muntah, mual ringan, gangguan gastrousus, hipotensi, anoreksia dan cirit-birit.

RAWATAN (untuk maklumat pengamal perubatan)

- Kosongkan perut melalui induksi pemberian emesis@ lavaj gastrik
- Pemberian *acetylcysteine* disyorkan segera setelah pengambilan berlebihan telah dilaporkan tanpa menunggu keputusan ujian air kencing dan ujian plasma. *Acetylcysteine* paling berkesan jika rawatan dimulakan dalam 10 hingga 12 jam setelah pengambilan berlebihan. Walau bagaimanapun, kemungkinan ada manfaatnya jika rawatan dimulakan dalam 24 jam. Untuk pemberian oral, dos *acetylcysteine* dewasa yang disyorkan adalah 140mg/kg pada mulanya, kemudian 70mg/kg setiap 4 jam selama 17 dos. Setiap dos harus dicairkan ke larutan cairan minum 5% sebelum diberikan kerana bau yang tidak menyenangkan dan sifat *sclerosing*. Mana-mana dos yang muntah dalam masa 1 jam selepas pemberian mesti diulang. Sekiranya perlu, penawar boleh diberikan melalui intubasi *duodenal*.
- Kepekatan plasma acetaminophen harus ditentukan sekurang-kurangnya 4 jam setelah pengambilan overdosis untuk memastikan status pesakit. Fungsi ginjal dan jantung harus dipantau dan terapi yang perlu dilaksanakan jika perlu.

Maklumat terkini mengenai rawatan overdosis boleh didapati dari Pusat Racun Nasional, Universiti Sains Malaysia, nombor telefon bimbit: 1-800-88-8099 (Isnin hingga Jumaat: 8.30 pagi - 4.30 petang, Sabtu: 8.30 pagi - 1.00 petang).

INTERAKSI DENGAN UBAT LAIN

Ubat yang mendorong isoenzim mikrosomal khusus yang bertanggungjawab daripada pengaktifan metabolik paracetamol mungkin dijangka meningkat kerentanan terhadap kesan toksiknya pada hati manusia seperti dalam kes di haiwan.

Penyerapan paracetamol boleh dipercepatkan oleh ubat-ubatan seperti metoclopramide. Perkumuhan mungkin terjejas dan kepekatan plasma diubah apabila diberi probenesid. Colestyramine mengurangkan penyerapan paracetamol jika diberikan dalam masa 1 jam. Interaksi ubat juga dilaporkan dengan warfarin, rifampicin, kloramfenikol, antiepileptik (carbamazepine, phenorbital, phenytoin, atau primidone), antiviral (interferon alfa dan zidovudine) dan probenecid.

KEHAMILAN DAN PENYUSUAN

Paracetamol boleh melintasi plasenta tetapi ubat itu telah digunakan secara meluas sebagai analgesik semasa kehamilan dan tiada kesan buruk kepada janin.

PEMBUNGKUSAN

Dalam botol 60ml dan 120ml.

PENYIMPANAN

PASTIKAN BEKAS DITUTUP RAPAT.

SIMPAN DI TEMPAT KERING DI BAWAH 30°C.

LINDUNGI DARI CAHAYA.

JAUHI DARIPADA KANAK-KANAK

JANGKA HAYAT

Sila rujuk pada kotak atau label.



Product Registration Holder:

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

Lot No. 2,4,6,8 & 10, Jalan P/7, Section 13,
Bangi Industrial Estate, 43650 Bandar Baru Bangi,
Selangor, Malaysia.

Manufacturer:

DUOPHARMA MANUFACTURING (BANGI) SDN. BHD.

Lot No. 2 & 4, Jalan P/7, Section 13, Bangi Industrial Estate,
43650 Bandar Baru Bangi, Selangor, Malaysia.

Revision Date: Feb 2026

1500008926 N.1

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Black 100%